

Prior Authorization Request - Inpatient Admission

Health Plan

Humana Medicare Advantage. Plan ID: H5216-014-000. Request type: standard pre-service review.
Date of request: 2026-10-04

Member Information

Member: Dennis Kowalczyk DOB: 1955-12-14 (age 70) Member ID: MBR-30619
Address: 630 Harbor View Dr, Lorain, OH 44052
Phone: (440) 555-0174 Sex: M
Primary insurance: Humana Medicare Advantage (primary). Secondary: none on file.

Requesting Provider

Dr. Helen Abernathy, MD -- Cardiac Electrophysiology
NPI: 1417258093
Practice: Erie Shores Heart Institute
Phone: (440) 555-0190 Fax: (440) 555-0191

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)
CPT 33249 ICD-10: I25.5 (Ischemic cardiomyopathy); I50.22
Planned admit date: 2026-10-14
Level of care requested: INPATIENT
Place of service: Hospital, inpatient (acute care).

Submission Contact

Submitted through the provider portal by Wanda Oyelaran, Authorization Coordinator.
Callback: (440) 555-0190. Supporting clinical documentation attached (8 pages including this request).

Cardiology Consultation - ICD Evaluation

Date: 2026-09-29. Patient: Dennis Kowalczyk Age: 70 Sex: M

Reason for referral

Primary-prevention ICD evaluation, ischemic cardiomyopathy with reduced ejection fraction.

History of present illness

70-year-old man with long-standing multivessel coronary artery disease managed medically, and ischemic cardiomyopathy with chronic systolic heart failure. He reports dyspnea on walking up a slope or more than two blocks, with no symptoms during routine household tasks and none at rest (NYHA II). No syncope. No sustained ventricular arrhythmia documented.

He was hospitalized earlier this month; see the interval history below and the discharge summary on page 6.

Interval history

Admitted 2026-09-09 to Erie Shores Medical Center with chest pressure and a troponin rise, diagnosed with an NSTEMI. Coronary angiography the next day showed known multivessel disease without a target for PCI; managed medically. Discharged 2026-09-12 free of angina.

PMH

Coronary artery disease, ischemic cardiomyopathy, hypertension, hyperlipidemia, type 2 diabetes (metformin). No CABG. No prior stent. Former smoker, quit 2015. Left inguinal hernia repair 2012.

Exam

BP 124/70, HR 66, SpO2 96%. Lungs with minimal bibasilar crackles. No edema. Regular rhythm.

Assessment and plan

Ischemic cardiomyopathy, LVEF 31%, NYHA II, status post recent NSTEMI. Planned ICD per EP. SDM visit completed (page 7). Continue GDMT and antiplatelet therapy.

Echocardiogram Report

Study date: 2026-09-11 (inpatient, post-NSTEMI day 2). Type: limited TTE.

Patient: Dennis Kowalczyk. Reader: C. Hoffmeyer, MD.

Findings

Left ventricle dilated, LVEDD 6.1 cm. Akinesis of the basal-to-mid inferior and inferolateral segments with hypokinesis of the anterior wall. Quantitative LVEF 31% (biplane Simpson).

Mild-to-moderate ischemic mitral regurgitation. RV function preserved. LA mildly dilated. No LV thrombus. No pericardial effusion.

Impression

Ischemic cardiomyopathy with LVEF 31% by echocardiography. Regional wall-motion abnormalities in the RCA/LCx territory.

Cardiac Catheterization Report

Date: 2026-09-10. Indication: NSTEMI, troponin I peak 14.2 ng/mL. Operator: R. Ingram, MD.

Angiographic findings

Left main: 30% distal stenosis. LAD: diffuse 60-70% mid disease, small caliber distal vessel. LCx: 80% proximal OM1 stenosis, diffusely diseased distal vessel. RCA: chronic total occlusion proximal, collateralized from the left system. Presumed culprit: subtotal OM1 lesion with small territory.

Decision

Heart team review: no suitable target for PCI given diffuse small-vessel disease; CABG not recommended at this time (poor distal targets, patient preference). Medical management intensified. No stent deployed.

Hemodynamics

LVEDP 24 mmHg. Aortic 128/74.

Heart Failure Medication List with Start Dates

Patient: Dennis Kowalczyk. Reconciled 2026-09-29.

Drug	Dose	Start date
sacubitril/valsartan	49/51 mg BID	2025-08-04
metoprolol succinate	100 mg daily	2025-08-04
epplerenone	25 mg daily	2025-09-15
dapagliflozin	10 mg daily	2025-10-20
furosemide	40 mg daily	2025-08-04
aspirin	81 mg daily	2012
clopidogrel	75 mg daily	2026-09-10 (post NSTEMI)
rosuvastatin	40 mg daily	2026-09-10 (increased)
metformin	1000 mg BID	2016

Patient has been on guideline-directed HF therapy for more than a year (14 months since 2025-08).

Hospital Discharge Summary (Erie Shores Medical Center)

Patient: Dennis Kowalczyk. Admitted: 2026-09-09. Discharged: 2026-09-12.

Principal diagnosis

Non-ST elevation myocardial infarction (NSTEMI), onset 2026-09-09.

Hospital course

Presented with 2 hours of substernal chest pressure. ECG with lateral ST depression, troponin I peak 14.2 ng/mL. Started on heparin, dual antiplatelet therapy and high-intensity statin. Cath 2026-09-10: multivessel disease, no PCI or CABG possible (see page 4). Limited echo 2026-09-11 with LVEF 31%. Remained hemodynamically stable, no recurrent chest pain, no arrhythmia on telemetry.

Disposition

Discharged home 2026-09-12 in stable condition, NYHA II. Follow up in cardiology within 2 weeks. ICD evaluation as an outpatient.

Shared Decision Making Visit Note - ICD

Date: 2026-09-30. Clinician: Dr. H. Abernathy (EP). Duration 35 minutes. Patient and daughter present.

We used the ICD patient decision aid (illustrated pamphlet and interactive web tool, evidence-based) to review the survival benefit of a primary-prevention ICD, procedural risk, shocks, and the alternative of medical therapy alone. The patient described his own priorities and chose to move ahead with the device. Informed of the recent heart attack and the usual waiting period; patient states understanding.

Outcome

Shared decision making visit completed and documented before implant scheduling.

Electrophysiology Surgeon Operative Plan

Date: 2026-09-30. Surgeon: Dr. Helen Abernathy, MD, cardiac electrophysiology. Patient: Dennis Kowalczyk.

Procedure

Transvenous ICD (single chamber), CPT 33249.

Indication

Primary prevention, ischemic cardiomyopathy with LVEF 31%, NYHA class II.

Expected stay

2 midnights, telemetry and device interrogation, pocket check.

Comorbidities

Coronary artery disease, heart failure, type 2 diabetes, hypertension, hyperlipidemia.

Status

Clinically stable since discharge on 09-12. No other conditions limiting survival; no atrial fibrillation; no severe neurologic impairment.